

The home-health nurse is caring for a 2-month-old client.

➤ Drag words from the choices below to fill in each blank in the following sentence.

The nurse should anticipate that the physician will instruct the parent to

Word Choice

 and

Word Choice

 .

Submit

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The nurse should anticipate that the physician will instruct the parent to

fortify the breast milk

 and

complete a feeding log

.

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CORRECT

Score: 1 / 1 point

Rationale:

Across 3 weekly visits, the client's weight has trended steadily downward despite frequent (6 to 8 times daily) breastfeeding on demand, now totaling a 12.5% weight loss, alongside worsening lethargy -- this is failure to thrive from inadequate caloric intake. Because breastfeeding on demand makes true intake volumes difficult to quantify, the physician would first have the parent complete a feeding log to objectively track how much and how often the client is feeding. At the same time, fortifying the breast milk (adding measured calories per ounce) increases caloric density while preserving the benefits of breastfeeding, a less drastic first step than replacing breastfeeding with formula altogether. Feeding the client formula for 2 weeks is a bigger change in feeding method than this trend supports as a first step, since fortifying breast milk has not yet been tried. Increasing the parent's own caloric intake does not directly address the infant's documented weight loss. Consulting a pediatric surgeon for gastrostomy tube placement is a far more invasive intervention reserved for failure to thrive that does not respond to less invasive measures, not an appropriate next step after only 3 visits.

Question 1

NOT COMPLETE

The nurse in the pediatric unit is caring for a 6-month-old client.

Nurses' Notes

0800: Client admitted with increased irritability and a leaking gastrostomy feeding tube, which was placed 2 weeks ago for failure to thrive. The feeding tube insertion site, which is on the left side of the abdomen, is covered with a dressing that is saturated with old formula. On removal of the dressing, the skin surrounding the feeding tube site is erythematous and flaking, and the feeding tube is loose. At the insertion site, a small amount of thick, yellow drainage is noted. Peripheral pulses are weak. Extremities are cool to the touch. Capillary refill is 3 seconds. Client is intermittently pulling at the tube and scratching at the site. Parent reports giving the client acetaminophen last night before bedtime, but the client was still intermittently waking and irritable throughout the night. Parent attempted to feed the client through the feeding tube 8 hours ago. Vital signs: temporal T 100.6° F (38.1° C), P 171, RR 42, BP 74/62, pulse oximetry reading 97% on room air. Parent has a history of a penicillin allergy.

The nurse is reviewing the client's assessment data to prepare the client's plan of care.

- Complete the diagram by dragging from the choices below to specify what condition the client is most likely experiencing, 2 actions the nurse should take to address that condition, and 2 parameters the nurse should monitor to assess the client's progress.

Action to Take

Action to Take

Condition Most Likely Experiencing

Parameter to Monitor

Parameter to Monitor

Actions to Take	Potential Conditions	Parameters to Monitor
change the site dressing	refeeding syndrome	stool output
request a wound consultation	infection of the gastrostomy tube site	skin integrity
obtain an electrocardiogram (ECG)	normal gastrostomy tube findings	feeding tolerance
request a bolus of intravenous 0.9% sodium chloride (normal saline)	intolerance to gastrostomy tube feedings	vital signs every 30 minutes
reassure the parent that the site findings are the normal progression of healing		parent's ability to administer a tube feeding

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CORRECT

Score: 5 / 5 points

Rationale: